

Executive Summary

The uploaded workbook is a **team-level AHT leaderboard**, not raw interaction-level WFM data. It contains:

- **47 team or queue rows**
- **697 agents represented**
- **39 valid current-AHT rows**
- Monthly AHT averages from **August through June**
- Current goals, achievement percentages, agent counts, and qualitative notes

It does **not** contain daily dates, intervals, call volume, ASA, ACW, hold time, transfers, occupancy, adherence, site, shift, call type, or individual agent identifiers. Therefore, those dimensions cannot be analyzed reliably from this file.

The main conclusions are:

- Current AHT averages **396.0 seconds unweighted** and **440.7 seconds weighted by agent count**
- Median current AHT is **385.3 seconds**
- AHT ranges from **238.5 to 562.2 seconds**
- **56.4% of teams are above their current AHT goal**
- The highest-risk areas are concentrated in **Breast Health, Core 2, Spanish, RIA, and DIN**
- The five largest excess-AHT contributors account for approximately **63.9% of the weighted excess-AHT index**
- The strongest available relationship is between current AHT and recent historical AHT, especially June AHT with a Spearman correlation of **0.875**
- The monthly trend is not statistically significant, with an unweighted trend p-value of **0.418**
- A 10% AHT reduction would create an illustrative capacity benefit of approximately **69.7 FTE-equivalents**, assuming agent count is a reasonable staffing proxy

Overall AHT profile

Metric	Result
Unweighted average	396.0 seconds
Agent-count-weighted average	440.7 seconds
Median	385.3 seconds
Minimum	238.5 seconds
Maximum	562.2 seconds
Standard deviation	87.7 seconds
Valid team rows	39
Agents represented	697

The weighted average is materially higher than the unweighted average because larger teams tend to have higher AHT. This means the operational experience of the workforce is closer to **441 seconds** than to the simple row average of **396 seconds**.

Key Findings Prioritized by Impact

1. Excess AHT is concentrated in a small number of queues

The largest contributors to excess AHT above goal are:

- Breast Health NE
 - 70 agents
 - Current AHT of 480.0 seconds
 - 30.0 seconds above goal
- Core 2 NE
 - 90 agents
 - Current AHT of 461.5 seconds
 - 21.5 seconds above goal
- Breast Health MA
 - 40 agents
 - Current AHT of 434.1 seconds
 - 34.1 seconds above goal
- Full Spanish
 - 41 agents
 - Current AHT of 540.4 seconds
 - 20.4 seconds above goal
- RR Spanish Core 1-2
 - 16 agents
 - Current AHT of 562.2 seconds
 - 42.2 seconds above goal

Together, these five teams contribute approximately **63.9% of the weighted excess-AHT index** across the rows with goals. This is the clearest prioritization signal in the workbook.

2. Spanish and Elite Specialty have the highest sustained AHT levels

The highest current-AHT teams are:

- RR Spanish Core 1-2 at **562.2 seconds**
- MultiModality Global at **557.6 seconds**
- Full Spanish at **540.4 seconds**
- Elite Specialty NE at **521.9 seconds**
- Elite Specialty MA at **519.6 seconds**

At LOB level, the highest agent-weighted AHT averages are:

- MMSP at **557.6 seconds**
- Spanish at **526.0 seconds**
- Elite Specialty at **521.0 seconds**
- Core 2 at **471.9 seconds**
- Breast Health at **458.3 seconds**

These areas should not necessarily be compared directly against low-complexity queues. However, the fact that several of them are also above goal and have low achievement rates makes them the highest operational-risk segments.

3. More than half of teams are above goal

Of the 39 valid current-AHT rows:

- **22 teams**, or **56.4%**, are above current goal
- Above-goal teams exceed target by an average of **25.2 seconds**
- Teams at or below goal are **18.7 seconds below target on average**

The largest goal gaps are:

- RIA Core 1 at **+92.8 seconds**
- DIN Core 2 at **+59.0 seconds**
- Elite 1 NE at **+54.3 seconds**
- RR Spanish Core 1-2 at **+42.2 seconds**
- Breast Health MA at **+34.1 seconds**

This suggests that goal attainment is not a marginal issue restricted to one isolated team.

4. Large teams matter more than extreme small teams

Some small teams have very high AHT, but the largest staffing impact comes from large teams with moderately high AHT.

For example:

- Core 2 NE has **90 agents** and is **21.5 seconds above goal**
- Breast Health NE has **70 agents** and is **30.0 seconds above goal**
- Breast Health MA has **40 agents** and is **34.1 seconds above goal**

These teams should generally be addressed before very small teams such as RIA Core 1, even though RIA Core 1 has a larger per-row gap.

Monthly and Trend Analysis

The available monthly data covers August through June. There is no daily or interval timestamp, so daily, weekly, intraday, and interval trends cannot be calculated.

Monthly pattern

The unweighted monthly averages were:

- August at **409.0 seconds**
- January at **396.7 seconds**
- March at **414.0 seconds**
- April at **424.8 seconds**
- June at **396.6 seconds**

The pattern shows:

- A gradual decline from August through January
- A rebound in March and April
- A return toward lower levels in June

Relative to August:

- January was approximately **3.0% lower**
- April was approximately **3.9% higher**
- June was approximately **3.0% lower**

The agent-weighted trend tells a slightly different story because higher-headcount groups carry more weight. The weighted average increased in April and remained elevated in June, reaching approximately **444.4 seconds** in June.

However, the linear monthly trend is not statistically significant:

- Unweighted slope of **+0.74 seconds per month**
- Unweighted p-value of **0.418**
- Weighted slope of **+1.49 seconds per month**
- Weighted p-value of **0.190**

This means the workbook supports a **short-term fluctuation story**, not a proven long-term upward or downward trend.

Current versus June

The current weighted AHT is approximately **0.8% lower than the June weighted average**. That is directionally positive, but too small to describe as a major structural improvement.

The largest current increases relative to June include:

- RIA Core 1 at **+41.4%**
- NWR at **+13.1%**
- BII Core 2 at **+10.1%**
- Chinese at **+9.7%**
- Elite 1 NE at **+8.5%**

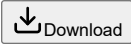
The largest decreases include:

- PNM Admin at **-40.9%**
- Spanish at **-14.1%**
- RIA Core 2 at **-7.7%**
- Elite 2 MA at **-6.3%**

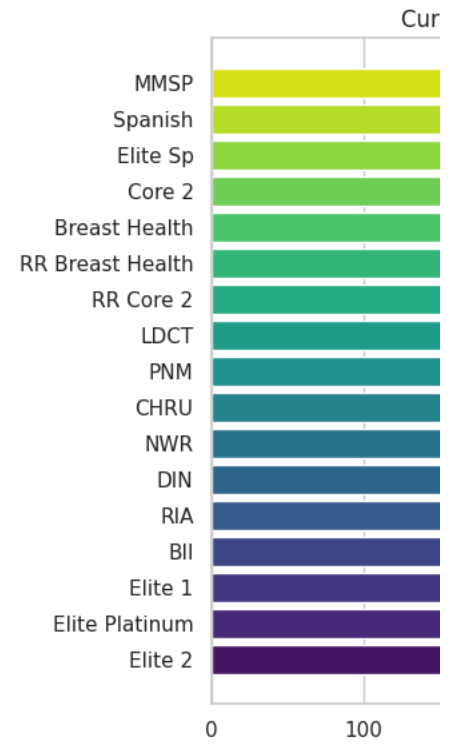
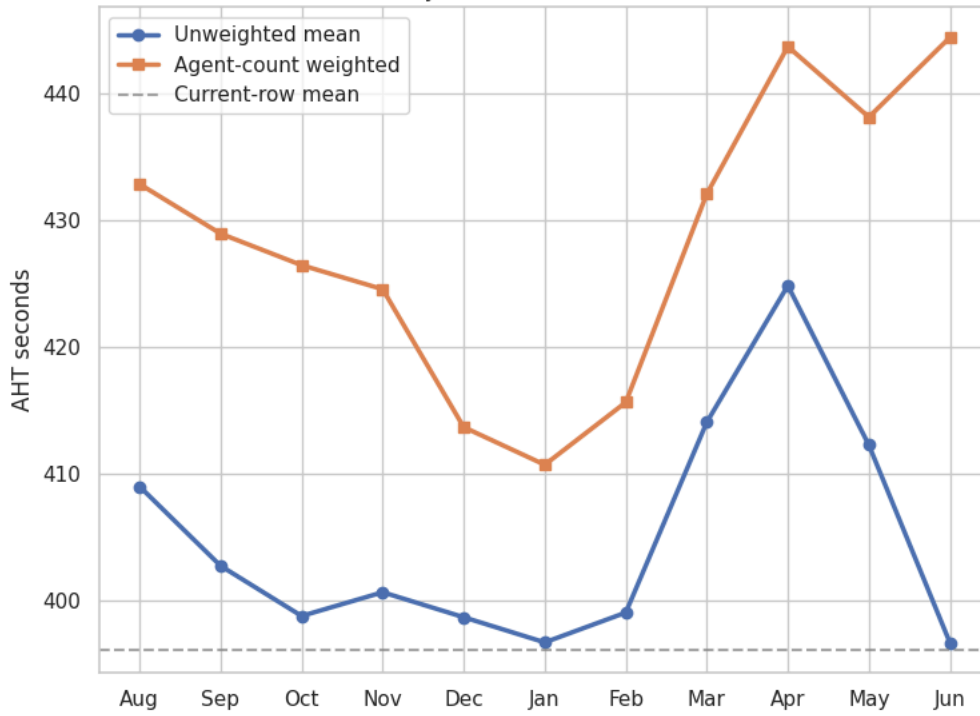
Small-team percentage changes should be interpreted cautiously because a few agents can move the team average substantially.

Charts and Visualizations

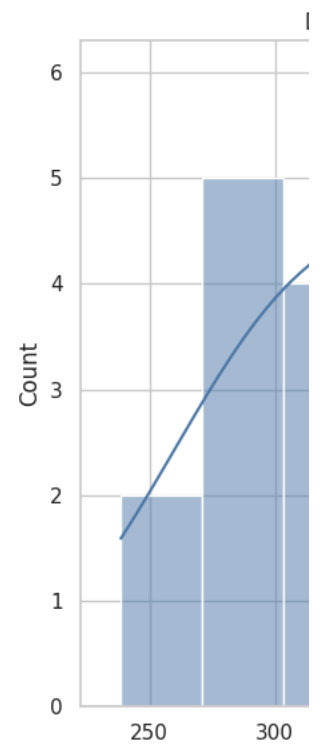
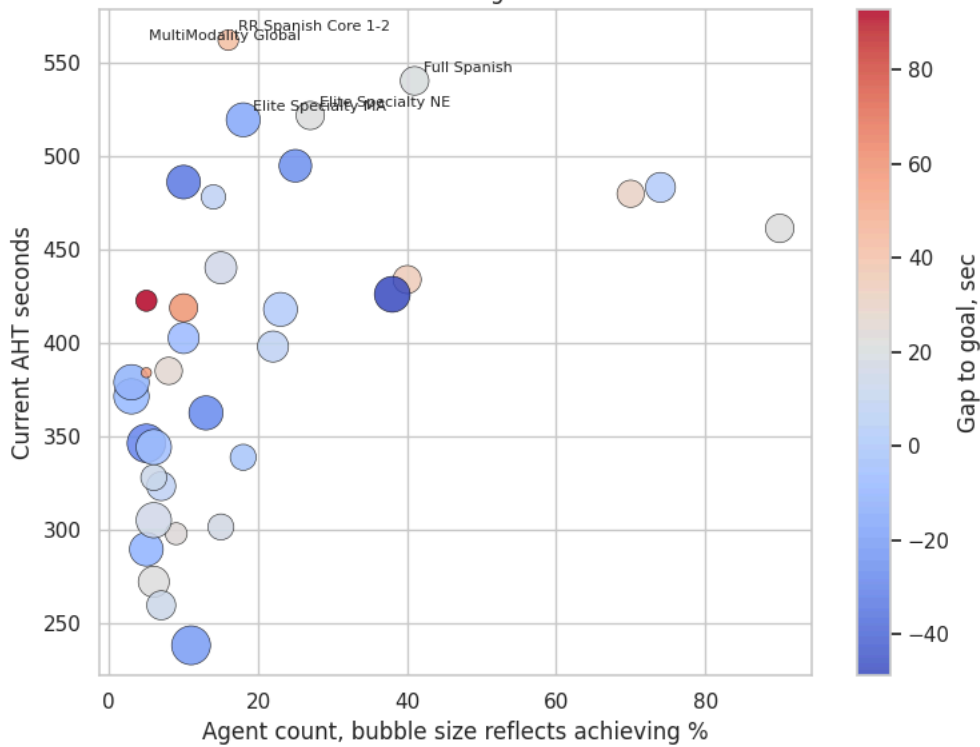
AHT trends, LOB comparison, distribution, and team-size relationship



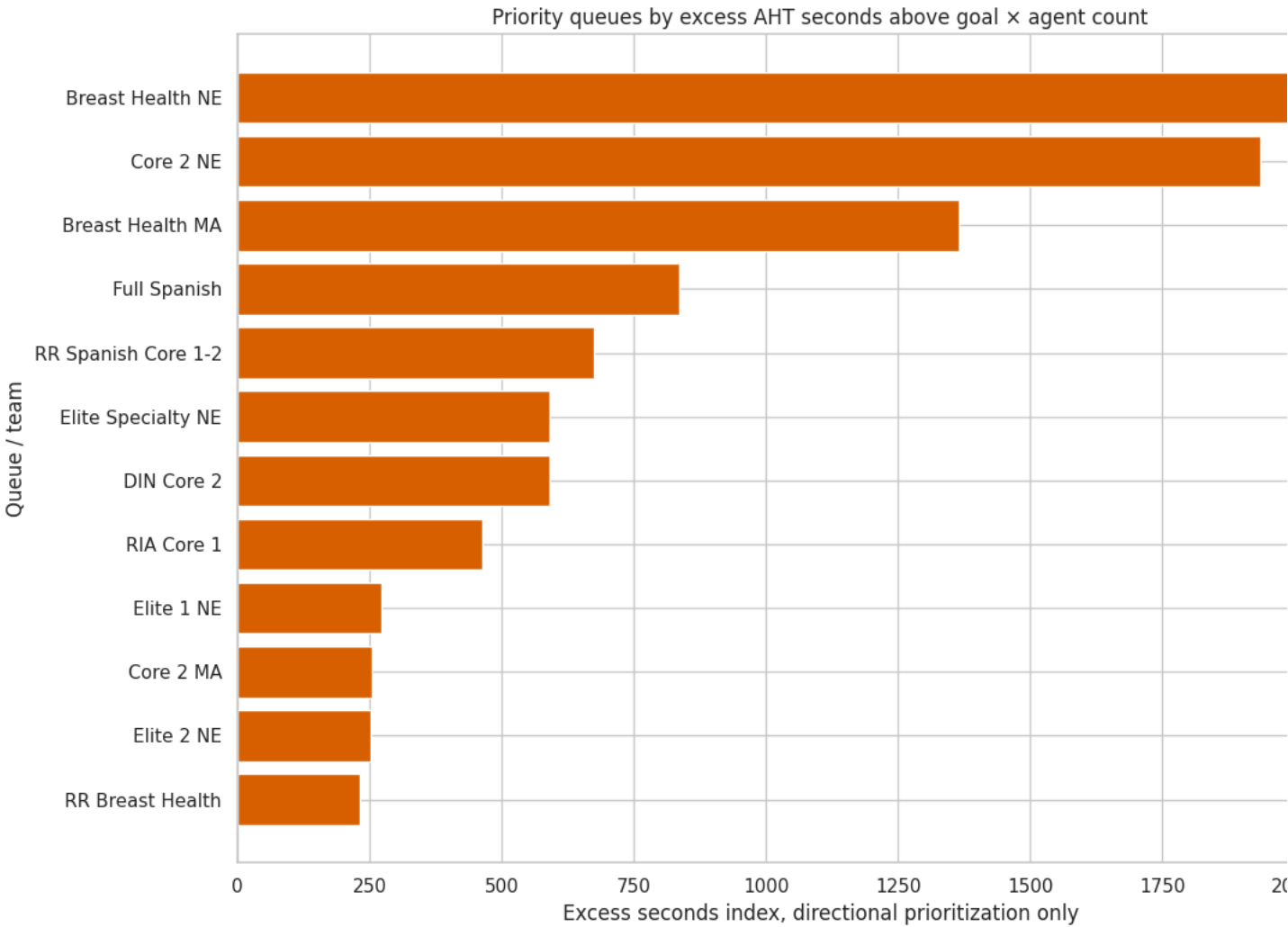
Monthly AHT trend in the workbook



Current AHT vs agent count



Excess-AHT prioritization



The second chart is a directional prioritization tool. It multiplies the positive gap to goal by agent count. It is not a direct workload measure because contact volume is not available, but it helps identify where AHT improvement could have the largest staffing impact.

LOB-Level Findings

The LOB results are summarized below.

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No rows to display

Highest-risk LOBs

Spanish

- Weighted current AHT of approximately **526.0 seconds**
- 92 agents represented
- Current AHT is near or above the 520-second goal depending on queue
- Highest-risk team is RR Spanish Core 1-2 at **562.2 seconds**
- Full Spanish is also elevated at **540.4 seconds**

Elite Specialty

- Weighted current AHT of approximately **521.0 seconds**
- 45 agents represented
- Elite Specialty NE is above goal by **21.9 seconds**
- Elite Specialty MA is below its 535-second goal but still has high absolute AHT at **519.6 seconds**

Core 2

- Weighted current AHT of approximately **471.9 seconds**
- 178 agents represented
- This is one of the most important capacity opportunities because it is the largest represented LOB
- Core 2 NE contributes a large excess-AHT index because it combines high AHT with 90 agents

Breast Health

- Weighted current AHT of approximately **458.3 seconds**
- 120 agents represented
- Breast Health NE and Breast Health MA together are major contributors to excess seconds above goal

MMSP

- Current AHT of approximately **557.6 seconds**
- Only four agents represented
- High AHT, but relatively limited staffing impact at current headcount

Top and Bottom Performers

Lowest current AHT

The lowest current-AHT teams are:

- Elite 2 MA at **238.5 seconds**
- Elite 2 BII at **259.9 seconds**
- BII Core 1 at **272.4 seconds**
- Elite 1 MA at **289.8 seconds**
- PNM Admin at **292.4 seconds**
- DIN Core 1 at **298.2 seconds**

These teams may provide useful process or workflow benchmarks, but their results should not be copied blindly into higher-complexity queues.

Highest current AHT

The highest current-AHT teams are:

- RR Spanish Core 1-2 at **562.2 seconds**
- MultiModality Global at **557.6 seconds**
- Full Spanish at **540.4 seconds**
- Elite Specialty NE at **521.9 seconds**
- Elite Specialty MA at **519.6 seconds**
- Spanish at **495.0 seconds**
- Core 2 MA at **483.4 seconds**
- Breast Health NE at **480.0 seconds**

The management-priority table is shown below.

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	LOB	Queue_or_Team	Agent_Count	Current_AHT	Current_Goal	Goal_Gap_sec	Achieving_Pct	Current_vs_June_pct	Notes
40	RIA	RIA Core 1	5	422.78	330.0	92.78	0.2	0.41	Fluctuates a lot-keep aligned with C2.
14	DIN	DIN Core 2	10	419.0	360.0	59.0	0.4	0.04	Keep an eye on-may need to inc soon. Inc to 37...
17	Elite 1	Elite 1 NE	5	384.29	330.0	54.29	0.0	0.09	
33	Spanish	RR Spanish Core 1-2	16	562.22	520.0	42.22	0.19	0.01	
5	Breast Health	Breast Health MA	40	434.12	400.0	34.12	0.4	0.02	A lot of new hires-do not increase too much
6	Breast Health	Breast Health NE	70	480.02	450.0	30.02	0.37	0.01	
3	BII	BII Core 2	8	385.25	360.0	25.25	0.38	0.1	One outlier dragging down average.
13	DIN	DIN Core 1	9	298.15	275.0	23.15	0.22	0.05	Inc to 280?
2	BII	BII Core 1	6	272.44	250.0	22.44	0.5	0.03	
24	Elite Sp	Elite Specialty NE	27	521.9	500.0	21.9	0.41	-0.01	
11	Core 2	Core 2 NE	90	461.5	440.0	21.5	0.41	-0.01	
31	Spanish	Full Spanish	41	540.41	520.0	20.41	0.41	0.08	

	LOB	Queue_or_Team	Agent_Count	Current_AHT	Current_Goal	Goal_Gap_sec	Achieving_Pct	Current_vs_June_pct	Notes
22 >	Elite 2	Elite 2 NE	15	301.79	285.0	16.79	0.33	0.02	Backing up ES - consider making exceptions if t...
46 >	RR Breast Health	RR Breast Health	15	440.44	425.0	15.44	0.53	-0.02	
21 >	Elite 2	Elite 2 NE LHR	6	305.38	290.0	15.38	0.67	0.05	Probably drop soon-285ish

Hidden Patterns

High AHT is strongly persistent over time

Current AHT has strong positive relationships with recent historical AHT:

- Current AHT versus March AHT
 - Spearman rho of **0.782**
- Current AHT versus June AHT
 - Spearman rho of **0.875**

This indicates that high-AHT teams are generally not experiencing a one-month anomaly. They tend to remain high over time.

The operational implication is that the best intervention targets are likely to be **structural queue characteristics, workflow design, training, call mix, language complexity, or escalation patterns**, rather than isolated performance events.

Agent count is associated with higher AHT

The relationship between agent count and current AHT is moderately positive:

- Spearman rho of **0.532**

This may indicate that larger teams are serving more complex or diverse workloads. It could also reflect a mix effect where large groups include multiple call types, skill levels, or hiring cohorts.

This should not be interpreted as larger teams causing higher AHT. The relationship is descriptive and could be confounded by queue complexity.

Achievement percentage is not a strong standalone predictor of AHT

Current AHT and achieving percentage have a weak negative relationship:

- Spearman rho of **-0.207**

This is important. A team can have high AHT but still achieve a relatively high percentage if the goal is appropriately calibrated for complexity. Conversely, a lower-AHT team can have poor achievement if its goal is set very aggressively.

AHT should therefore be evaluated together with:

- Goal calibration
- Call complexity
- Volume
- Service level
- Quality
- Customer effort
- Occupancy and staffing

Several notes point to workforce mix and process complexity

The workbook notes provide useful root-cause hypotheses:

- New hires affecting AHT
- New workflows requiring more time
- CMI-related complexity
- High-dollar calls
- Gradual movement into MultiModality calls
- Agents receiving Core 2 goals
- Fluctuating performance in RIA
- An outlier affecting BII Core 2

These notes suggest that the dominant drivers are likely to include **ramp maturity, call mix transitions, specialized workflows, and cross-queue movement**.

Root Cause Analysis

The dataset does not include ASA, ACW, hold time, transfer rate, occupancy, adherence, volume, or call-level classifications, so causal decomposition cannot be completed numerically. The strongest root-cause hypotheses from the available data are below.

New-hire and ramp effects

Several notes explicitly mention new hires. This is consistent with elevated AHT in:

- Breast Health MA at **434.1 seconds**
- Elite Specialty MA at **519.6 seconds**
- Core 2 MA at **483.4 seconds**
- PNM and other teams with recent staffing changes

Recommended diagnostic cuts include:

- Tenure bands
- New-hire versus tenured AHT
- AHT by nesting stage
- AHT by training cohort
- AHT after independent production begins

Call-complexity mix

The highest AHT segments are concentrated in specialized or multilingual work:

- Spanish
- MultiModality
- Elite Specialty
- Core 2
- Breast Health

These may have longer documentation requirements, more clinical complexity, more research, more transfers, or more complex customer interactions.

The correct test is to decompose AHT into:

- Talk time
- Hold time
- After-call work
- Transfer-related time
- Call type
- Skill
- Escalation or consultation indicators

Workflow and tooling

Notes refer to:

- New workflows
- CMI
- High-dollar calls
- MultiModality call movement

These are likely process drivers rather than pure agent-efficiency issues. Before applying performance pressure, the operation should validate:

- Number of required systems
- Screen navigation time
- Documentation requirements
- Knowledge-base usability
- Decision-tree complexity
- Approval or escalation steps
- Repeat contacts caused by incomplete resolution

Goal calibration

Some teams are below goal but still have high absolute AHT. For example, Elite Specialty MA is at **519.6 seconds** against a 535-second goal. That is technically favorable against target but still operationally expensive.

This supports a two-dimensional view:

- **Goal performance**
- **Absolute AHT and capacity impact**

Both are needed for workforce planning.

Business Impact Estimate

Because contact volume is not available, a direct cost or service-level calculation cannot be made. The estimates below are **capacity-equivalent scenarios**, using the 697 represented agents as a staffing proxy.

Illustrative AHT reduction scenarios

Scenario	Weighted AHT	Capacity gain	FTE-equivalent
5% AHT reduction	418.7 sec	5.3%	34.9
10% AHT reduction	396.6 sec	11.1%	69.7
15% AHT reduction	374.6 sec	17.6%	104.6

These figures do **not** mean that the operation can immediately remove that number of employees. They represent potential capacity that could be redirected toward:

- Absorbing more demand

- Reducing overtime
- Improving service level
- Reducing occupancy pressure
- Supporting shrinkage
- Covering hiring delays
- Reducing backlog

A more defensible financial estimate would require:

- Contact volume
- Interval arrival pattern
- Required service level
- Occupancy target
- Shrinkage
- Fully loaded labor cost
- Actual talk, hold, and ACW components

Actionable Recommendations

Quick wins

Prioritize the five largest excess-AHT contributors

Focus first on:

- Breast Health NE
- Core 2 NE
- Breast Health MA
- Full Spanish
- RR Spanish Core 1-2

These teams provide the highest likely return because they combine meaningful AHT gaps with material agent counts.

Run targeted call listening

Review a statistically representative sample of calls from:

- RIA Core 1
- DIN Core 2
- RR Spanish Core 1-2
- Breast Health NE
- Core 2 NE

Tag each call for:

- Hold events
- Transfers
- Repeat explanations
- Escalations
- Tool delays
- Documentation burden
- Knowledge gaps
- Complex call types

Separate new-hire effects from process effects

Create a simple tenure split:

- 0 to 30 days
- 31 to 60 days
- 61 to 90 days
- 90-plus days

This will show whether high AHT is expected ramp behavior or a persistent process issue.

Protect low-AHT benchmarks

Review Elite 2 MA, Elite 1 MA, and BII Core 1 to identify practices that may be transferable, while controlling for call complexity.

Medium-term actions

Decompose AHT into its components

Add reporting for:

- Talk time
- Hold time
- ACW
- Transfers
- Consults
- Escalations
- Repeat contacts
- Call type

This is essential because the correct intervention differs by component. For example:

- High talk time suggests conversation flow or complexity
- High hold time suggests system or knowledge issues
- High ACW suggests documentation burden
- High transfer rates suggest routing or skill gaps

Create a complexity-adjusted AHT target

Use separate targets for:

- Language
- Skill
- Call type
- New versus tenured agents
- Specialty queue
- Escalation status

This will reduce the risk of penalizing teams for serving inherently more complex work.

Review staffing alignment in large high-AHT teams

Core 2 NE and Breast Health NE have large agent populations and elevated AHT. They should be assessed for:

- Skill mix
- Schedule alignment
- Occupancy
- Shrinkage
- Coaching capacity
- Queue spillover
- Cross-trained agent utilization

Long-term actions

Build a driver-based AHT model

Once the necessary fields are available, model AHT using:

- Queue
- LOB
- Skill
- Site
- Shift
- Agent tenure
- Call type
- Volume
- ASA
- Occupancy
- Adherence
- Hold
- ACW
- Transfers
- Quality
- Recontacts

Use a mixed-effects or hierarchical model so team and agent effects can be separated from call-mix effects.

Forecast AHT and staffing together

AHT forecasting should be integrated with:

- Arrival volume
- Service-level target
- Occupancy
- Shrinkage
- Schedule efficiency
- Staffing plans
- Hiring and nesting timelines

AHT alone is not sufficient for an accurate FTE forecast.

Establish statistical process-control monitoring

For each major queue, monitor:

- Weekly mean and median AHT
- Control limits
- Percentile AHT
- New-hire cohort performance
- Goal attainment
- Component-level drivers

This will distinguish true process changes from normal variation.

Final Assessment

The operation has a clear AHT opportunity, but the opportunity is **concentrated rather than universal**. The most effective strategy is not a broad AHT reduction mandate. It is a targeted program focused on:

- Breast Health NE and MA
- Core 2 NE
- Spanish queues
- RIA Core 1
- DIN Core 2
- New-hire and workflow-related drivers

The current workbook is strong enough to prioritize queues and estimate directional capacity impact. It is not sufficient to determine whether high AHT is caused by ASA, ACW, hold time, transfers, occupancy, adherence, call volume, or agent behavior. Those fields should be added before making individual-agent decisions or claiming causal improvement.